Cal Poly Pomona

College of Business Administration
SINGELYN GRADUATE SCHOOL OF BUSINESS

CEO Business Challenge

Analyzing Health-Care Service Models for Inland Empire Health Plan (IEHP)

Info Session October 7, 2026



Agenda

- **Welcome**
- **Challenge Overview**
- **Timeline**
- **Company Background**
- **The Business Problem**
- **Deliverables**
- **Website and Registration link**
- **Q and A**



CEO Business Challenge 2026

Your Ideas Could Shape the Future of Health Care

Join the CEO Business Challenge with Inland Empire Health Plan (IEHP): analyze real-world health care data and present your ideas to industry leaders.

Open to all CPP students. Join as a team of 3–5 or register individually. No health care experience required.

Info session: October 7, 2026, 5:30–6:30 PM • Building 163, Room 1015

Register by: October 16, 2026

Final presentations: November 19, 2026, 1:00–3:30 PM • Building 163, Room 1005

Event Details and Registration

Digital Badges

- Earn a digital badge to showcase your achievement CEO Business Challenge 2026 Presenter



- Compete for cash prizes awarded to 1st, 2nd, and 3rd place winners CEO Business Challenge 2026 Distinguished Achievement



Timeline

Wednesday Oct 7, 5:30 PM – 6:30 PM; 163-1015	Info Session; in-person
Wednesday Oct 14, 5:30 PM – 6:30 PM; 164-1080	Networking Session with Jayson Yardley, CEO, Patient Funding Alternatives
Friday Oct 16	Registration Closes
Monday Oct 19	Teams formed from individual registrations

Timeline

Monday Oct 19	Case document, selected data files, and video guidelines posted to Canvas
Monday Oct 19 – Sunday Nov 8	Three week challenge period
Monday Nov 9	Five minute video presentations due
Friday Nov 13	Finalist teams announced
Thursday Nov 19, 1:00 PM – 3:30 PM; 163-1005 (Lunch provided before start of event)	Final presentations, Awards, Networking

Company Background

- IEHP was founded in 1996, organized as a not-for-profit health plan serving residents of **Riverside and San Bernardino counties**
- Membership of **~1.4 million**
- One of the top 5 largest **Medicaid health plans** and the largest not-for-profit **Medicare-Medicaid** public health plan in the country
- Accreditation from **National Committee for Quality Assurance** and Certified as a **Great Place to Work**
- Company website is https://www.iehp.org/



Health Care: The "Big Picture"

- One of the largest sectors in the US economy ~ **20% of GDP**
- **Everyone is a healthcare consumer**
- One of the most **heavily regulated**, a single regulatory decision can have **huge impacts**
- **Medi-Cal** serves ~ **one-third of all Californians** (14 million) through two primary delivery models
  - Managed Care Plans (MCP)
  - Fee-for-Service (FFS)
- IEHP covers **over 90% of the Medi-Cal managed care market in the Inland Empire**

The Business Problem – Challenge Context

- **Changes in health care policy for Medi-Cal** will take effect from January 1, 2027

- California Department of Health Care Services (DHCS) plans to transition **~ 2 million Medi-Cal members** with **Unsatisfactory Immigration Status (UIS)** from **Managed Care Plans to Fee-for-Service**



Key Differences Between Managed Care and Fee-for-Service

Domain	Managed Care	Fee-for-Service
Provider Choice and Access	Members **access** care through **providers in the MCP's network**. Members select or are assigned a **primary care provider (PCP).** MCPs are **required to maintain** an adequate network to provide medically necessary covered services.	Members may see any provider statewide who is enrolled as a Medi-Cal FFS provider and is accepting Medi-Cal FFS members; **no assigned PCP**. **Access depends on provider participation in FFS**, not just coverage.
Care Coordination	PCP is **responsible for delivering primary care services**, making referrals, leading care management, and **coordinating services** with a care management team as needed.	**Limited** centralized care coordination. Some providers may bill for care coordination services. This may **affect continuity of care** and patient navigation.
Provider Management	**MCP manages** provider credentialing, authorizations, utilization management, and pays providers.	Providers work directly with **DHCS** for enrollment, authorizations (when required), and claims payment.
Community Supports & Enhanced Care Management (ECM)	**Available** through Managed Care.	**Not available** under FFS. Some members may lose access to services that address social and complex care needs.

Stakeholders

- State of California
- **IEHP**
- Provider
- **Patient**



Problem Statement

- **Impact on IEHP**: What does this shift mean for IEHP **financially** and **operationally** (for example, lost revenue, fixed costs, and its provider network)? What are the potential **risks** and unintended **consequences**?

- **Impact on patients**: Where might patients experience the greatest **loss of access** to care? **Which geographic areas** appear most vulnerable because of limited provider participation?

- **Actions**: What should **IEHP prioritize** in the short and long term?

- **Metrics:** What **should IEHP monitor** during and after the transition?

Possible Angles of Impacts for IEHP

- **Operational**
  - Continuity of care
  - Provider participation
  - Patient navigation
  - Access to specialty services

Strategic

- Financial
- Network
- Risk pooling



Possible Angles of Impacts for IEHP

- **Financial**
  - **Revenue loss:** State pays capitation fees to IEHP (fixed amount per member per month PMPM)
    - Different for different groups
    - According to IEHP Finance Committee report , Medi-Cal PMPM is estimated around $500
    - How many IEHP members will be impacted?
  - **Implications for fixed costs:**
    - E.g., Staffing, IT, finance

Background Research and Data Sources

- IEHP website
  - Fact sheet



  - Governing board reports (including finance committee reports)

- DHCS Policy Guide for UIS

- Additional data sources will be shared on Oct 19

Deliverables

- **Analyze** the problem in **teams of 3-5 students**
- Prepare a **five-minute video presentation** with your analysis and insights
  - Due **Monday Nov 9**
  - Focus on **executive-level storytelling supported by data**
  - Provide **recommendations to the CEO** on how he should respond to the transition
- **Finalists** will be invited to **present on Thursday Nov 19**



Guidelines

- **Step into the role**
- Feel free to make **reasonable assumptions** and **model what-if scenarios**
- **Limited use of Generative AI** is permitted (see next slide)
  - Please remember that you are responsible for the accuracy of your analysis and findings
- **Collaborate and enjoy!**



Use of Generative AI

- **Yes:** Summarizing articles and policy documents, brainstorming ideas, help with Excel formulas or code, editing your script
- **No:** AI-generated numbers you can’t trace to a source, AI voices or avatars in place of your team presenting
- Every number in your video must come from a source you can name
- Add one final slide listing the AI tools you used and what you used them for

Networking Session



- Jayson Yardley, **CEO**, Patient Funding Alternatives
- **Leader** and **healthcare expert**
- “The choice for this year's CEO Business Challenge is timely, highly relevant, and gives students the opportunity to work on a **complex problem** that has **real implications** for patients, providers, and the broader **healthcare system**”
- Learn how healthcare executives **approach complex strategic decisions** and what CEOs typically look for in **executive presentations**
- Wednesday Oct 14, 5:30 PM – 6:30 PM, 164-1080

Website with Registration Link

- https://www.cpp.edu/cba/ceo-business-challenge/

Q and A

Thank you!



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